



PATIENT NAME : JITENDRA SINGH

REF. DOCTOR : SELF

CODE/NAME & ADDRESS : C000132123

HCL AVITAS PVT LTD

806 SIDDARTHA, 96 NEHRU PLACE,SOUTH DELHI

NEW DELHI 110019

9910998037

ACCESSION NO : 0009YI010587

PATIENT ID : H1.HN2-12903

CLIENT PATIENT ID: H1.HN2-12903

ABHA NO :

AGE/SEX : 44 Years Male

DRAWN : 09/09/2025 07:19:40

RECEIVED : 09/09/2025 14:10:25

REPORTED : 09/09/2025 17:30:41

Test Report Status **Final**

Results

Biological Reference Interval Units

HAEMATOLOGY - CBC

NEWGEN - HOME VISIT PACKAGE

BLOOD COUNTS,EDTA WHOLE BLOOD

HEMOGLOBIN (HB)	15.0	13.0 - 17.0	g/dL
METHOD : SPECTROPHOTOMETRY			
RED BLOOD CELL (RBC) COUNT	4.85	4.5 - 5.5	mil/ μ L
METHOD : IMPEDANCE			
WHITE BLOOD CELL (WBC) COUNT	5.25	4.0 - 10.0	thou/ μ L
METHOD : IMPEDANCE			
PLATELET COUNT	191	150 - 410	thou/ μ L
METHOD : IMPEDANCE			

RBC AND PLATELET INDICES

HEMATOCRIT (PCV)	44.7	40 - 50	%
METHOD : CALCULATED			
MEAN CORPUSCULAR VOLUME (MCV)	92.1	83 - 101	fL
METHOD : DERIVED FROM IMPEDANCE MEASURE			
MEAN CORPUSCULAR HEMOGLOBIN (MCH)	30.8	27.0 - 32.0	pg
METHOD : CALCULATED PARAMETER			
MEAN CORPUSCULAR HEMOGLOBIN CONCENTRATION (MCHC)	33.5	31.5 - 34.5	g/dL
METHOD : CALCULATED PARAMETER			
RED CELL DISTRIBUTION WIDTH (RDW)	13.5	11.6 - 14.0	%
METHOD : DERIVED FROM IMPEDANCE MEASURE			
MEAN PLATELET VOLUME (MPV)	11.0 High	6.8 - 10.9	fL
METHOD : DERIVED FROM IMPEDANCE MEASURE			

WBC DIFFERENTIAL COUNT

NEUTROPHILS	56	40 - 80	%
METHOD : DHSS FLOWCYTOMETRY			
LYMPHOCYTES	28	20 - 40	%
METHOD : DHSS FLOWCYTOMETRY			
MONOCYTES	10	2 - 10	%
METHOD : DHSS FLOWCYTOMETRY			

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Haryana, India
Tel : 9111591115, Fax : CIN - U74899PB1995PLC045956



ULR No.9000014734570-0009

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EOSINOPHILS		06	1 - 6	%
METHOD : DHSS FLOWCYTOMETRY				
BASOPHILS		00	0 - 2	%
METHOD : IMPEDANCE				
ABSOLUTE NEUTROPHIL COUNT		2.94	2.0 - 7.0	thou/ μ L
METHOD : DHSS FLOWCYTOMETRY, CALCULATED				
ABSOLUTE LYMPHOCYTE COUNT		1.46	1 - 3	thou/ μ L
METHOD : DHSS FLOWCYTOMETRY, CALCULATED				
ABSOLUTE MONOCYTE COUNT		0.52	0.20 - 1.00	thou/ μ L
METHOD : DHSS FLOWCYTOMETRY, CALCULATED				
ABSOLUTE EOSINOPHIL COUNT		0.32	0.02 - 0.50	thou/ μ L
METHOD : DHSS FLOWCYTOMETRY, CALCULATED				
ABSOLUTE BASOPHIL COUNT		0	0.00 - 0.10	thou/ μ L
METHOD : DHSS FLOWCYTOMETRY, CALCULATED				

PS(PERIPHERAL SMEAR EXAM,EDTA WHOLE BLOOD

RBC

PREDOMINANTLY NORMOCYTIC NORMOCHROMIC

METHOD : MICROSCOPIC EXAMINATION

WBC

NORMAL IN NUMBER, MORPHOLOGY AND DISTRIBUTION

METHOD : MICROSCOPIC EXAMINATION

PLATELETS

NORMAL IN NUMBER AND MORPHOLOGY.

METHOD : MICROSCOPIC EXAMINATION

ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD

E.S.R

9

0 - 10

mm at 1 hr

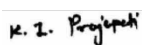
METHOD : AUTOMATED (PHOTOMETRICAL CAPILLARY STOPPED FLOW KINETIC ANALYSIS)

Interpretation(s)

BLOOD COUNTS,EDTA WHOLE BLOOD-The cell morphology is well preserved for 24hrs. However after 24-48 hrs a progressive increase in MCV and HCT is observed leading to a decrease in MCHC. A direct smear is recommended for an accurate differential count and for examination of RBC morphology.

RBC AND PLATELET INDICES-Mentzer index (MCV/RBC) is an automated cell-counter based calculated screen tool to differentiate cases of Iron deficiency anaemia(>13) from Beta thalassaemia trait (<13) in patients with microcytic anaemia. This needs to be interpreted in line with clinical correlation and suspicion. Estimation of HbA2 remains the gold standard for diagnosing a case of beta thalassaemia trait.

WBC DIFFERENTIAL COUNT-The optimal threshold of 3.3 for NLR showed a prognostic possibility of clinical symptoms to change from mild to severe in COVID positive



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patients. When age = 49.5 years old and NLR = 3.3, 46.1% COVID-19 patients with mild disease might become severe. By contrast, when age < 49.5 years old and NLR < 3.3, COVID-19 patients tend to show mild disease.

(Reference to - The diagnostic and predictive role of NLR, d-NLR and PLR in COVID-19 patients A.-P. Yang, et al. International Immunopharmacology 84 (2020) 106504 This ratio element is a calculated parameter and out of NABL scope.

ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD-TEST DESCRIPTION :- Erythrocyte sedimentation rate (ESR) is a test that indirectly measures the degree of inflammation present in the body. The test actually measures the rate of fall (sedimentation) of erythrocytes in a sample of blood that has been placed into a tall, thin, vertical tube. Results are reported as the millimetres of clear fluid (plasma) that are present at the top portion of the tube after one hour. Nowadays fully automated instruments are available to measure ESR.

- ESR is not diagnostic it is a non-specific test that may be elevated in a number of different conditions. It provides general information about the presence of an inflammatory condition.CRP is superior to ESR because it is more sensitive and reflects a more rapid change.

TEST INTERPRETATION : Increase in: Infections, Vasculitis, Inflammatory arthritis, Renal disease, Anemia, Malignancies and plasma cell dyscrasias, Acute allergy Tissue injury, Pregnancy, Estrogen medication, Aging.

Finding a very accelerated ESR(>100 mm/hour) in patients with ill-defined symptoms directs the physician to search for a systemic disease (Paraproteinemias, Disseminated malignancies, connective tissue disease, severe infections such as bacterial endocarditis).

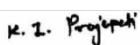
In pregnancy BRI in first trimester is 0-48 mm/hr(62 if anemic) and in second trimester (0-70 mm /hr(95 if anemic). ESR returns to normal 4th week post partum.

Decreased in: Polycythemia vera, Sickle cell anemia

LIMITATIONS : False elevated ESR : Increased fibrinogen, Drugs(Vitamin A, Dextran etc), Hypercholesterolemia

False Decreased : Poikilocytosis,(SickleCells,spherocytes),Microcytosis, Low fibrinogen, Very high WBC counts, Drugs(Quinine, salicylates)

REFERENCE : Nathan and Oski's Haematology of Infancy and Childhood, 5th edition 2. Paediatric reference intervals. AACC Press, 7th edition. Edited by S. Soldin 3. The reference for the adult reference range is "Practical Haematology by Dacie and Lewis,10th edition.



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HAEMATOLOGY

NEWGEN - HOME VISIT PACKAGE

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD

HBA1C

5.1

Non-diabetic: < 5.7 %

Pre-diabetics: 5.7 - 6.4

Diabetics: > or = 6.5

ADA Target: 7.0

Action suggested: > 8.0

METHOD : HIGH PERFORMANCE LIQUID CHROMATOGRAPHY

ESTIMATED AVERAGE GLUCOSE(EAG)

99.7

< 116

mg/dL

METHOD : CALCULATED PARAMETER

Interpretation(s)

GLYCOSYLATED HEMOGLOBIN(HBA1C),EDTA WHOLE BLOOD-Used For:

1. Evaluating the long-term control of blood glucose concentrations in diabetic patients.

2. Diagnosing diabetes.

3. Identifying patients at increased risk for diabetes (prediabetes).

The ADA recommends measurement of HbA1c (typically 3-4 times per year for type 1 and poorly controlled type 2 diabetic patients, and 2 times per year for well-controlled type 2 diabetic patients) to determine whether a patient's metabolic control has remained continuously within the target range.

1. eAG (Estimated average glucose) converts percentage HbA1c to mg/dl, to compare blood glucose levels.

2. eAG gives an evaluation of blood glucose levels for the last couple of months.

3. eAG is calculated as eAG (mg/dl) = 28.7 * HbA1c - 46.7

HbA1c Estimation can get affected due to :

1. Shortened Erythrocyte survival : Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g. recovery from acute blood loss,hemolytic anemia) will falsely lower HbA1c test results.Fructosamine is recommended in these patients which indicates diabetes control over 15 days.

2.Vitamin C & E are reported to falsely lower test results.(possibly by inhibiting glycation of hemoglobin.

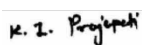
3. Iron deficiency anemia is reported to increase test results. Hypertriglyceridemia,uremia, hyperbilirubinemia, chronic alcoholism,chronic ingestion of salicylates & opiates addition are reported to interfere with some assay methods,falsely increasing results.

4. Interference of hemoglobinopathies in HbA1c estimation is seen in

a) Homozygous hemoglobinopathy. Fructosamine is recommended for testing of HbA1c.

b) Heterozygous state detected (D10 is corrected for HbS & HbC trait.)

c) HbF > 25% on alternate platform (Boronate affinity chromatography) is recommended for testing of HbA1c.Abnormal Hemoglobin electrophoresis (HPLC method) is recommended for detecting a hemoglobinopathy



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BIOCHEMISTRY

NEWGEN - HOME VISIT PACKAGE

GLUCOSE FASTING,FLUORIDE PLASMA

FBS (FASTING BLOOD SUGAR)

96

(Normal <100, Impaired fasting glucose: 100 to 125, Diabetes mellitus: ≥ 126 (on more than 1 occasion) (ADA guidelines 2024))

METHOD : SPECTROPHOTOMETRY HEXOKINASE

LIVER FUNCTION PROFILE, SERUM

BILIRUBIN, TOTAL

0.7

Upto 1.2

mg/dL

METHOD : COLORIMETRIC DIAZO METHOD

BILIRUBIN, DIRECT

0.3

< or = 0.3

mg/dL

METHOD : COLORIMETRIC DIAZO METHOD

BILIRUBIN, INDIRECT

0.40

0.1 - 1.0

mg/dL

METHOD : CALCULATED PARAMETER

TOTAL PROTEIN

7.3

6.0 - 8.0

g/dL

METHOD : SPECTROPHOTOMETRY, BIURET

ALBUMIN

4.3

3.97 - 4.94

g/dL

METHOD : SPECTROPHOTOMETRY, BROMOCRESOL GREEN (BCG) - DYE BINDING

GLOBULIN

3.0

2.0 - 3.5

g/dL

METHOD : CALCULATED PARAMETER

ALBUMIN/GLOBULIN RATIO

1.4

1.0 - 2.1

RATIO

METHOD : CALCULATED PARAMETER

ASPARTATE AMINOTRANSFERASE (AST/SGOT)

32

< OR = 50

U/L

METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC

ALANINE AMINOTRANSFERASE (ALT/SGPT)

47

< OR = 50

U/L

METHOD : SPECTROPHOTOMETRY, WITH PYRIDOXAL PHOSPHATE ACTIVATION-IFCC

ALKALINE PHOSPHATASE

96

40 - 129

U/L

METHOD : SPECTROPHOTOMETRY, PNPP, AMP BUFFER - IFCC

GAMMA GLUTAMYL TRANSFERASE (GGT)

21

0 - 60

U/L

METHOD : ENZYMATIC COLORIMETRIC ASSAY STANDARDIZED AGAINST IFCC / SZASZ

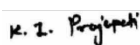
LACTATE DEHYDROGENASE

164

125 - 220

U/L

METHOD : SPECTROPHOTOMETRY, LACTATE TO PYRUVATE - UV-IFCC

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Consultant - PathologistDr. Rashmi Rasi Datta-MD, FIMSA
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BLOOD UREA NITROGEN (BUN), SERUM

BLOOD UREA NITROGEN	9.1	6 - 20	mg/dL
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METHOD : SPECTROPHOTOMETRY, KINETIC TEST WITH UREASE AND GLUTAMATE DEHYDROGENASE

CREATININE, SERUM

CREATININE	1.28	0.90 - 1.30	mg/dL
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METHOD : SPECTROPHOTOMETRIC, JAFFE'S KINETICS

URIC ACID, SERUM

URIC ACID	6.7	3.4 - 7.0	mg/dL
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METHOD : SPECTROPHOTOMETRY, URICASE

CALCIUM, SERUM

CALCIUM	9.3	8.6 - 10.0	mg/dL
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METHOD : SPECTROPHOTOMETRY, NM - BAPTA

IRON, SERUM

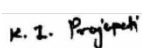
IRON	79	59 - 158	µg/dL
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METHOD : SPECTROPHOTOMETRY, FERROZINE

PHOSPHORUS, SERUM

PHOSPHORUS	3.4	2.5 - 4.5	mg/dL
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METHOD : SPECTROPHOTOMETRY, MOLYBDATE UV METHOD



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GLUCOSE FASTING, FLUORIDE PLASMA-TEST DESCRIPTION

Normally, the glucose concentration in extracellular fluid is closely regulated so that a source of energy is readily available to tissues and so that no glucose is excreted in the urine.

Increased in: Diabetes mellitus, Cushing's syndrome (10 – 15%), chronic pancreatitis (30%). Drugs: corticosteroids, phenytoin, estrogen, thiazides.

Decreased in: Pancreatic islet cell disease with increased insulin, insulinoma, adrenocortical insufficiency, hypopituitarism, diffuse liver disease, malignancy (adrenocortical, stomach, fibrosarcoma), infant of a diabetic mother, enzyme deficiency diseases (e.g. galactosemia), Drugs-insulin, ethanol, propranolol, sulfonylureas, tolbutamide, and other oral hypoglycemic agents.

NOTE: While random serum glucose levels correlate with home glucose monitoring results (weekly mean capillary glucose values), there is wide fluctuation within individuals. Thus, glycosylated hemoglobin (HbA1c) levels are favored to monitor glycemic control.

High fasting glucose level in comparison to post prandial glucose level may be seen due to effect of Oral Hypoglycaemics & Insulin treatment, Renal Glycosuria, Glycaemic index & response to food consumed, Alimentary Hypoglycemia, Increased insulin response & sensitivity etc.

LIVER FUNCTION PROFILE, SERUM-

Bilirubin is a yellowish pigment found in bile and is a breakdown product of normal heme catabolism. Bilirubin is excreted in bile and urine, and elevated levels may give yellow discoloration in jaundice. **Elevated levels** results from increased bilirubin production (eg, hemolysis and ineffective erythropoiesis), decreased bilirubin excretion (eg, obstruction and hepatitis), and abnormal bilirubin metabolism (eg, hereditary and neonatal jaundice). Conjugated (direct) bilirubin is elevated more than unconjugated (indirect) bilirubin in Viral hepatitis, Drug reactions, Alcoholic liver disease. Conjugated (direct) bilirubin is also elevated more than unconjugated (indirect) bilirubin when there is some kind of blockage of the bile ducts like in Gallstones getting into the bile ducts, tumors & Scarring of the bile ducts. Increased unconjugated (indirect) bilirubin may be a result of Hemolytic or pernicious anemia, Transfusion reaction & a common metabolic condition termed Gilbert syndrome, due to low levels of the enzyme that attaches sugar molecules to bilirubin.

AST is an enzyme found in various parts of the body. AST is found in the liver, heart, skeletal muscle, kidneys, brain, and red blood cells, and it is commonly measured clinically as a marker for liver health. AST levels increase during chronic viral hepatitis, blockage of the bile duct, cirrhosis of the liver, liver cancer, kidney failure, hemolytic anemia, pancreatitis, hemochromatosis. AST levels may also increase after a heart attack or strenuous activity. ALT test measures the amount of this enzyme in the blood. ALT is found mainly in the liver, but also in smaller amounts in the kidneys, heart, muscles, and pancreas. It is commonly measured as a part of a diagnostic evaluation of hepatocellular injury, to determine liver health. AST levels increase during acute hepatitis, sometimes due to a viral infection, ischemia to the liver, chronic hepatitis, obstruction of bile ducts, cirrhosis.

ALP is a protein found in almost all body tissues. Tissues with higher amounts of ALP include the liver, bile ducts and bone. Elevated ALP levels are seen in Biliary obstruction, Osteoblastic bone tumors, osteomalacia, hepatitis, Hyperparathyroidism, Leukemia, Lymphoma, Paget's disease, Rickets, Sarcoidosis etc. Lower-than-normal ALP levels seen in Hypophosphatasia, Malnutrition, Protein deficiency, Wilson's disease.

GGT is an enzyme found in cell membranes of many tissues mainly in the liver, kidney and pancreas. It is also found in other tissues including intestine, spleen, heart, brain and seminal vesicles. The highest concentration is in the kidney, but the liver is considered the source of normal enzyme activity. Serum GGT has been widely used as an index of liver dysfunction. Elevated serum GGT activity can be found in diseases of the liver, biliary system and pancreas. Conditions that increase serum GGT are obstructive liver disease, high alcohol consumption and use of enzyme-inducing drugs etc.

Total Protein also known as total protein, is a biochemical test for measuring the total amount of protein in serum. Protein in the plasma is made up of albumin and globulin. Higher-than-normal levels may be due to: Chronic inflammation or infection, including HIV and hepatitis B or C, Multiple myeloma, Waldenström's disease. Lower-than-normal levels may be due to: Agammaglobulinemia, Bleeding (hemorrhage), Burns, Glomerulonephritis, Liver disease, Malabsorption, Malnutrition, Nephrotic syndrome, Protein-losing enteropathy etc.

Albumin is the most abundant protein in human blood plasma. It is produced in the liver. Albumin constitutes about half of the blood serum protein. Low blood albumin levels (hypoalbuminemia) can be caused by: Liver disease like cirrhosis of the liver, nephrotic syndrome, protein-losing enteropathy, Burns, hemodilution, increased vascular permeability or decreased lymphatic clearance, malnutrition and wasting etc.

BLOOD UREA NITROGEN (BUN), SERUM-Causes of Increased levels include Pre renal (High protein diet, Increased protein catabolism, GI haemorrhage, Cortisol, Dehydration, CHF Renal), Renal Failure, Post Renal (Malignancy, Nephrolithiasis, Prostatism)

Causes of decreased level include Liver disease, SIADH.

CREATININE, SERUM-Higher than normal level may be due to:

• Blockage in the urinary tract, Kidney problems, such as kidney damage or failure, infection, or reduced blood flow, Loss of body fluid (dehydration), Muscle problems, such as breakdown of muscle fibers, Problems during pregnancy, such as seizures (eclampsia), or high blood pressure caused by pregnancy (preeclampsia)

Lower than normal level may be due to: • Myasthenia Gravis, Muscular Dystrophy

URIC ACID, SERUM-Causes of Increased levels: Dietary (High Protein Intake, Prolonged Fasting, Rapid weight loss), Gout, Lesch Nyhan syndrome, Type 2 DM, Metabolic syndrome **Causes of decreased levels:** Low Zinc intake, OCP, Multiple Sclerosis

CALCIUM, SERUM-Common causes of decreased value of calcium (hypocalcemia) are chronic renal failure, hypomagnesemia and hypoalbuminemia.

Hypercalcemia (increased value of calcium) can be caused by increased intestinal absorption (vitamin D intoxication), increased skeletal reabsorption (immobilization), or a combination of mechanisms (primary hyperparathyroidism). Primary hyperparathyroidism and malignancy accounts for 90-95% of all cases of hypercalcemia.

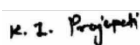
Values of total calcium is affected by serum proteins, particularly albumin thus, latter's value should be taken into account when interpreting serum calcium levels. The following regression equation may be helpful.

Corrected total calcium (mg/dl) = total calcium (mg/dl) + 0.8 (4 - albumin [g/dl])*

because regression equations vary among group of patients in different physiological and pathological conditions and mathematical corrections are only approximations.

The possible mathematical corrections should be replaced by direct determination of free calcium by ISE. A common and important source of preanalytical error in the measurement of calcium is prolonged tourniquet application during sampling. Thus, this along with fist clenching should be avoided before phlebotomy.

IRON, SERUM-Serum iron test is useful for etio- morphological diagnosis of anemias, in hemochromatosis, in hemosiderosis and in acute iron toxicity. Serum iron is recommended to be correlated with Total Iron Binding Capacity (TIBC) for evaluation of iron deficiency.



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ULR No. 9000014734570-0009

PATIENT NAME : JITENDRA SINGH

REF. DOCTOR : SELF

CODE/NAME & ADDRESS : C000132123

HCL AVITAS PVT LTD

806 SIDDARTHA, 96 NEHRU PLACE,SOUTH DELHI

NEW DELHI 110019

9910998037

ACCESSION NO : 0009YI010587

PATIENT ID : H1.HN2-12903

CLIENT PATIENT ID: H1.HN2-12903

ABHA NO :

AGE/SEX : 44 Years Male

DRAWN : 09/09/2025 07:19:40

RECEIVED : 09/09/2025 14:10:25

REPORTED : 09/09/2025 17:30:41

Test Report Status **Final**

Results

Biological Reference Interval Units

BIOCHEMISTRY - LIPID

NEWGEN - HOME VISIT PACKAGE

LIPID PROFILE WITH CALCULATED LDL,SERUM

CHOLESTEROL, TOTAL

213 High

Desirable : < 200 mg/dL

Borderline : 200 - 239

High : > / = 240

METHOD : ENZYMATIC COLORIMETRIC ASSAY

TRIGLYCERIDES

134

Normal: < 150 mg/dL

Borderline high: 150 - 199

High: 200 - 499

Very High: > / = 500

METHOD : ENZYMATIC COLORIMETRIC ASSAY

HDL CHOLESTEROL

42

At Risk: < 40 mg/dL

Desirable: > or = 60

METHOD : HOMOGENEOUS ENZYMATIC COLORIMETRIC ASSAY

CHOLESTEROL LDL

144 High

Adult levels: mg/dL

Optimal < 100

Near optimal/above optimal:

100-129

Borderline high : 130-159

High : 160-189

Very high : = 190

METHOD : CALCULATED PARAMETER

NON HDL CHOLESTEROL

171 High

Desirable : < 130 mg/dL

Above Desirable : 130 - 159

Borderline High : 160 - 189

High : 190 - 219

Very high : > / = 220

METHOD : CALCULATED PARAMETER

VERY LOW DENSITY LIPOPROTEIN

26.8

< OR = 30.0 mg/dL

METHOD : CALCULATED PARAMETER

CHOL/HDL RATIO

5.1 High

Low Risk : 3.3 - 4.4

Average Risk : 4.5 - 7.0

Moderate Risk : 7.1 - 11.0

High Risk : > 11.0

METHOD : CALCULATED PARAMETER

LDL/HDL RATIO

3.4 High

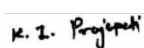
0.5 - 3.0 Desirable/Low Risk

3.1 - 6.0 Borderline/Moderate

Risk

>6.0 High Risk

METHOD : CALCULATED PARAMETER


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Interpretation(s)

Serum lipid profile is measured for cardiovascular risk prediction. Lipid Association of India recommends LDL-C as primary target and Non HDL-C as co-primary treatment target.

Risk Stratification for ASCVD (Atherosclerotic cardiovascular disease) by Lipid Association of India

Risk Category	
Extreme risk group	A. CAD with > 1 feature of high risk group
	B. CAD with > 1 feature of Very high risk group or recurrent ACS (within 1 year) despite LDL-C < or = 50 mg/dl or polyvascular disease
Very High Risk	1. Established ASCVD 2. Diabetes with 2 major risk factors or evidence of end organ damage 3. Familial Homozygous Hypercholesterolemia
High Risk	1. Three major ASCVD risk factors. 2. Diabetes with 1 major risk factor or no evidence of end organ damage. 3. CKD stage 3B or 4. 4. LDL >190 mg/dl 5. Extreme of a single risk factor. 6. Coronary Artery Calcium - CAC >300 AU. 7. Lipoprotein a >= 50mg/dl 8. Non stenotic carotid plaque
Moderate Risk	2 major ASCVD risk factors
Low Risk	0-1 major ASCVD risk factors
Major ASCVD (Atherosclerotic cardiovascular disease) Risk Factors	
1. Age > or = 45 years in males and > or = 55 years in females	3. Current Cigarette smoking or tobacco use
2. Family history of premature ASCVD	4. High blood pressure
5. Low HDL	

Newer treatment goals and statin initiation thresholds based on the risk categories proposed by LAI in 2020.

Risk Group	Treatment Goals		Consider Drug Therapy	
	LDL-C (mg/dl)	Non-HDL (mg/dl)	LDL-C (mg/dl)	Non-HDL (mg/dl)
Extreme Risk Group Category A	<50 (Optional goal < OR = 30)	< 80 (Optional goal <OR = 60)	>OR = 50	>OR = 80
Extreme Risk Group Category B	<OR = 30	<OR = 60	> 30	>60
Very High Risk	<50	<80	>OR= 50	>OR= 80
High Risk	<70	<100	>OR= 70	>OR= 100
Moderate Risk	<100	<130	>OR= 100	>OR= 130
Low Risk	<100	<130	>OR= 130*	>OR= 160

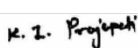
*After an adequate non-pharmacological intervention for at least 3 months.

References: Management of Dyslipidaemia for the Prevention of Stroke: Clinical Practice Recommendations from the Lipid Association of India. Current Vascular Pharmacology, 2022, 20, 134-155.

Interpretation of Diagnostic Tests by Jacques Wallach, 11th Ed.

Friedewald, W. T., Levy, R. I., & Fredrickson, D. S. (1972). "Estimation of the concentration of low-density lipoprotein cholesterol in plasma, without use of the preparative ultracentrifuge." Clinical Chemistry, 18(6), 499-502.

National Cholesterol Education Program (NCEP) Adult Treatment Panel III (ATP III). (2001).



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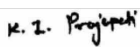
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Third Report of the National Cholesterol Education Program (NCEP) Expert Panel on Detection, Evaluation, and Treatment of High Blood Cholesterol in Adults (Adult Treatment Panel III) Final Report. Circulation, 106(25), 3143-3421.

American College of Cardiology (ACC) and American Heart Association (AHA) (2013). 2013 ACC/AHA Guideline on the Assessment of Cardiovascular Risk. Circulation, 129(25), S49-S73.



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NEPHELOMETRY

NEWGEN - HOME VISIT PACKAGE

RF FACTOR (RHEUMATOID FACTOR
QUANTITATIVE),SERUM

RHEUMATOID FACTOR

<10.0

< 14.0

IU/mL

METHOD : IMMUNOTURBIDIMETRIC ASSAY

Interpretation(s)

RF FACTOR (RHEUMATOID FACTOR QUANTITATIVE),SERUM-This test is used for diagnosis of Rheumatoid arthritis (RA) in individuals with a suggestive clinical presentation.

Rheumatoid factor is an IgM autoantibody directed against the Fc portion of Immunoglobulin G (IgG) and is found in more than two-thirds of adults with Rheumatoid arthritis. Detection of RF is one of the criteria of the American Rheumatology Association (ARA) for the diagnosis of Rheumatoid arthritis.

The presence of Rheumatoid factor is of prognostic significance also, since patients with high titres tend to have more severe and progressive disease.

RF is also found in a number of other conditions such as Systemic lupus erythematosus, Sjogren's syndrome, chronic liver disease, hepatitis B. It plays an important role in differential diagnosis between RA and other rheumatic diseases.

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Results

Biological Reference Interval Units

CLINICAL PATH - URINALYSIS

NEWGEN - HOME VISIT PACKAGE

PHYSICAL EXAMINATION, URINE

COLOR

PALE YELLOW

METHOD : MANUAL

APPEARANCE

CLEAR

METHOD : MANUAL

CHEMICAL EXAMINATION, URINE

PH

5.5

4.5 - 7.5

METHOD : DOUBLE INDICATOR PRINCIPLE

SPECIFIC GRAVITY

1.010

1.005 - 1.030

METHOD : IONIC CONCENTRATION METHOD/DIPSTICK

PROTEIN

NOT DETECTED

NOT DETECTED

METHOD : PROTEIN-ERROR-OF INDICATORS PRINCIPLE

GLUCOSE

NOT DETECTED

NEGATIVE

METHOD : PEROXIDASE / MANUAL

KETONES

NOT DETECTED

NOT DETECTED

METHOD : NITROPRUSSIDE REACTION / MANUAL

BLOOD

NOT DETECTED

NEGATIVE

METHOD : DISOPROPLBENZENE / MANUAL

BILIRUBIN

NOT DETECTED

NOT DETECTED

UROBILINOGEN

Nor

NORMAL

METHOD : DIETHYLAMINO BENZ ALDEHYDE / MANUAL

NITRITE

NOT DETECTED

NOT DETECTED

METHOD : DIAZONIUM COMPOUND METHOD

LEUKOCYTE ESTERASE

NOT DETECTED

NOT DETECTED

METHOD : ESTERASES/DIPSTICK

MICROSCOPIC EXAMINATION, URINE

RED BLOOD CELLS

NOT DETECTED

0 - 2

/HPF

METHOD : MICROSCOPY

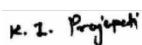
PUS CELL (WBCS)

0-1

0-5

/HPF

METHOD : MICROSCOPY



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Test Report Status	Final	Results	Biological Reference Interval	Units
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EPITHELIAL CELLS

0-1

0-5

/HPF

METHOD : MICROSCOPY

CASTS

NOT DETECTED

METHOD : MICROSCOPY

CRYSTALS

NOT DETECTED

METHOD : MICROSCOPY

BACTERIA

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPY

YEAST

NOT DETECTED

NOT DETECTED

METHOD : MICROSCOPY

Interpretation(s)

The following table describes the probable conditions, in which the analytes are present in urine

Presence of	Conditions
Proteins	Inflammation or immune illnesses
Pus (White Blood Cells)	Urinary tract infection, urinary tract or kidney stone, tumors or any kind of kidney impairment
Glucose	Diabetes or kidney disease
Ketones	Diabetic ketoacidosis (DKA), starvation or thirst
Urobilinogen	Liver disease such as hepatitis or cirrhosis
Blood	Renal or genital disorders/trauma
Bilirubin	Liver disease
Erythrocytes	Urological diseases (e.g. kidney and bladder cancer, urolithiasis), urinary tract infection and glomerular diseases
Leukocytes	Urinary tract infection, glomerulonephritis, interstitial nephritis either acute or chronic, polycystic kidney disease, urolithiasis, contamination by genital secretions
Epithelial cells	Urolithiasis, bladder carcinoma or hydronephrosis, ureteric stents or bladder catheters for prolonged periods of time
Granular Casts	Low intratubular pH, high urine osmolality and sodium concentration, interaction with Bence-Jones protein
Hyaline casts	Physical stress, fever, dehydration, acute congestive heart failure, renal diseases

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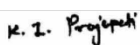
Results

Biological Reference Interval Units

Calcium oxalate	Metabolic stone disease, primary or secondary hyperoxaluria, intravenous infusion of large doses of vitamin C, the use of vasodilator naftidrofuryl oxalate or the gastrointestinal lipase inhibitor orlistat, ingestion of ethylene glycol or of star fruit (Averrhoa carambola) or its juice
Uric acid	arthritis
Bacteria	Urinary infection when present in significant numbers & with pus cells.
Trichomonas vaginalis	Vaginitis, cervicitis or salpingitis

Reference:

Henrys Clinical Diagnostics and Management by Laboratory Methods 21st edition (pg 410).



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Test Report Status Final**Results****Biological Reference Interval Units****SPECIALISED CHEMISTRY - HORMONE****NEWGEN - HOME VISIT PACKAGE****THYROID PANEL, SERUM**

T3 METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY	141.0	80 - 200	ng/dL
T4 METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY	7.81	5.1 - 14.1	µg/dL
TSH (ULTRASENSITIVE) METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY	5.090 High	0.270 - 4.200	µIU/mL

Interpretation(s)

Triiodothyronine T3, **Thyroxine T4**, and **Thyroid Stimulating Hormone TSH** are thyroid hormones which affect almost every physiological process in the body, including growth, development, metabolism, body temperature, and heart rate.

Production of T3 and its prohormone thyroxine (T4) is activated by thyroid-stimulating hormone (TSH), which is released from the pituitary gland. Elevated concentrations of T3, and T4 in the blood inhibit the production of TSH.

Excessive secretion of thyroxine in the body is hyperthyroidism, and deficient secretion is called hypothyroidism.

In primary hypothyroidism, TSH levels are significantly elevated, while in secondary and tertiary hyperthyroidism, TSH levels are low.

Below mentioned are the guidelines for Pregnancy related reference ranges for Total T4, TSH & Total T3. Measurement of the serum TT3 level is a more sensitive test for the diagnosis of hyperthyroidism, and measurement of TT4 is more useful in the diagnosis of hypothyroidism. Most of the thyroid hormone in blood is bound to transport proteins. Only a very small fraction of the circulating hormone is free and biologically active. It is advisable to detect Free T3, FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4.

Sr. No.	TSH	Total T4	FT4	Total T3	Possible Conditions
1	High	Low	Low	Low	(1) Primary Hypothyroidism (2) Chronic autoimmune Thyroiditis (3) Post Thyroidectomy (4) Post Radio-Iodine treatment
2	High	Normal	Normal	Normal	(1) Subclinical Hypothyroidism (2) Patient with insufficient thyroid hormone replacement therapy (3) In cases of Autoimmune/Hashimoto thyroiditis (4). Isolated increase in TSH levels can be due to Subclinical inflammation, drugs like amphetamines, Iodine containing drug and dopamine antagonist e.g. domperidone and other physiological reasons.
3	Normal/Low	Low	Low	Low	(1) Secondary and Tertiary Hypothyroidism
4	Low	High	High	High	(1) Primary Hyperthyroidism (Graves Disease) (2) Multinodular Goitre (3) Toxic Nodular Goitre (4) Thyroiditis (5) Over treatment of thyroid hormone (6) Drug effect e.g. Glucocorticoids, dopamine, T4 replacement therapy (7) First trimester of Pregnancy
5	Low	Normal	Normal	Normal	(1) Subclinical Hyperthyroidism



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6	High	High	High	High	(1) TSH secreting pituitary adenoma (2) TRH secreting tumor
7	Low	Low	Low	Low	(1) Central Hypothyroidism (2) Euthyroid sick syndrome (3) Recent treatment for Hyperthyroidism
8	Normal/Low	Normal	Normal	High	(1) T3 thyrotoxicosis (2) Non-Thyroidal illness
9	Low	High	High	Normal	(1) T4 Ingestion (2) Thyroiditis (3) Interfering Anti TPO antibodies

REF: 1. TIETZ Fundamentals of Clinical chemistry 2.Guidlines of the American Thyroid association during pregnancy and Postpartum, 2011.

TSH in pregnancy

There's reduction in both the lower and the upper limit of maternal TSH relative to the non-pregnant TSH reference range. This is because of elevated levels of serum hCG that directly stimulates the TSH receptor, thereby increasing thyroid hormone production. The largest decrease in serum TSH is observed during the first trimester. Thereafter, serum TSH and its reference range gradually increases in the second and third trimesters, but nonetheless remains lower than in non-pregnant women.

NOTE: It is advisable to detect Free T3,FreeT4 along with TSH, instead of testing for albumin bound Total T3, Total T4.TSH is not affected by variation in thyroid - binding protein. TSH has a diurnal rhythm, with peaks at 2:00 - 4:00 a.m. And troughs at 5:00 - 6:00 p.m. With ultradian variations.



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SPECIALISED CHEMISTRY - VITAMIN

NEWGEN - HOME VISIT PACKAGE

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM

25 - HYDROXYVITAMIN D

8.3 Low

Deficiency: < 20.0 ng/mL

Insufficiency: 20.0 - < 30.0

Sufficiency: 30.0 -100.0

Toxicity > 100.0

METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY

VITAMIN B12(CYANOCOBALAMINE), SERUM

VITAMIN B12

409

197 - 771

pg/mL

METHOD : ELECTROCHEMILUMINESCENCE IMMUNO ASSAY

Interpretation(s)

25 - HYDROXYVITAMIN D(VITAMIN D TOTAL),SERUM- Test description

Vitamin D has anti-inflammatory and immune-modulating properties and it works towards the bones, teeth, intestines, immune system, pancreas, muscles and brain. It helps to maintain normal calcium and phosphate levels. Vitamin D is a fat-soluble vitamin. Also called as "Sunshine Vitamin". Two main forms as Cholecalciferol (vitamin D3) which is synthesized in skin from 7-dehydrocholesterol in response to sunlight (Type B UV) exposure & Ergocalciferol (vitamin D2) present mainly in dietary sources.

Vit D25(OH)D deficiency is seen due to poor or inadequate sunlight exposure, Nutritional or dietary deficiency or fat malabsorption, Severe Hepatocellular disease, Secondary hyperparathyroidism, Hypocalcemia tetany which can cause involuntary contraction of muscles, leading to cramps and spasms, Rickets in children, Osteomalacia in adults- due to vitamin D deficiency mainly, Older adults- osteoporosis. (Increased risk of bone fractures) due to long-term effect of calcium and/or vitamin D deficiency, Other conditions that are precipitated by Vit D deficiency included increased cardiovascular risk, low immunity & chronic renal failure.

Elevated levels may be seen in patients taking supplements(hence recommended to repeat after 3 months for estimation of accurate levels), Vitamin D intoxication, sarcoidosis and malignancies containing non regulated 1-alpha hydroxylase in the lesion.

Recommendations

1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample 2.25(OH)D is the analyte of choice for determination of the Vitamin D status as it is the major storage & active form of Vitamin D and has longer half-life. 3. Kidney Disease Outcomes Quality Initiatives (KDOQI) and Kidney Disease Improving Global Outcomes (KDIGO) recommend activated vitamin D testing for CKD patients.

Note-Our Vitamin D assays is standardized to be in alignment with the ID-LC/MS/MS 25(OH)vitamin D Reference Method Procedure (RMP), the reference procedure for the Vitamin D Standardization Program (VDSP). The VDSP, a collaboration of the National Institutes of Health Office of Dietary Supplements, National Institute of Technology and Standards, Centers for Disease Control and Ghent University, is an initiative to standardize 25(OH)vitamin D measurement across methods.

Reference: 1.Wallach Interpretation of diagnostic test, 10th edition.

VITAMIN B12(CYANOCOBALAMINE), SERUM-Test description

1.Measures the amount of Vitamin B12/ Cyanocobalamin or Methyl cobalamin in blood.2. Done in Anemic conditions like Megaloblastic anemia, pernicious anemia, dietary folate deficiencies,3.Workup of neuropathies especially due to diabetes.4.Nerve health and it is monitored in treatment of nerve damage.5.Importance vitamin for women of childbearing age and for older people.

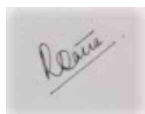
1.Part of water-soluble B complex of vitamins. 2. It is essential in DNA synthesis, hematopoiesis & CNS integrity.3.Source for B12 is dietary foods like milk, yoghurt, eggs, meat, fortified cereals, bread. 4.Absorption depends on the HCl secreted by the stomach and occurs in intestines. 5. It is part of enterohepatic circulation, hence excreted in feces(approx. 0.1% per day)

Test interpretation

Higher than normal levels are in patients on Vitamin supplements or patients with COPD, CRF, Diabetes, Liver cell damage, Obesity, Polycythemia.

Decreased levels seen in

Inflammatory bowel disease, Pernicious anemia - genetic deficiency of intrinsic factor - necessary for Vit B12 absorption, Strict vegetarians lead to sub-clinical B12 deficiency- high among elderly patients, Malabsorption due to gastrectomy, smoking, pregnancy, multiple myeloma & hemodialysis, Alcohol & drugs like amino salicylic acid, anticonvulsants, cholestyramine, cimetidine, Hyperthyroidism (High levels of thyroid), Seen in mothers of children with (NTD) Neural tube defects- hence fortification and



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PERFORMED AT :

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Reference Lab,2nd Floor, Plot No. 31,Urban Estate Electronic City,Sector-18,
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Tel : 9111591115, Fax : CIN - U74899PB1995PLC045956



ULR No.9000014734570-0009

PATIENT NAME : JITENDRA SINGH

REF. DOCTOR : SELF

CODE/NAME & ADDRESS : C000132123

HCL AVITAS PVT LTD

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9910998037

ACCESSION NO : 0009YI010587

PATIENT ID : H1.HN2-12903

CLIENT PATIENT ID: H1.HN2-12903

ABHA NO :

AGE/SEX : 44 Years Male

DRAWN : 09/09/2025 07:19:40

RECEIVED : 09/09/2025 14:10:25

REPORTED : 09/09/2025 17:30:41

Test Report Status **Final**

Results

Biological Reference Interval Units

supplements are advised in expecting mothers

Recommendations-1.To prevent biotin interference the patient should be atleast 8 hours fasting before submitting the sample. 2. Vit B12 and Folic acid evaluated together in macrocytic anemias to avoid methyl folate trap. Carmel's composite criteria for inadequate Vit B12 status: Serum vitamin B12 < 148 pmol/L, or 148-258 pmol/L and MMA > 0.30µmol/L, or tHcy > 13 nmol/L (females) and >15 nmol/L (males).

Associated Test-Holo-TC: Marker of vitamin B12 status -specificity and sensitivity better than serum vitamin B12, hence recommended in borderline and deficient cases for confirmation.

References-O-Leary F, Samman S. Vitamin B12 in health and disease. Nutrients. 2010 Mar 2(3):299-316.

****End Of Report****Please visit www.agilusdiagnostics.com for related Test Information for this accession**CONDITIONS OF LABORATORY TESTING & REPORTING**

1. It is presumed that the test sample belongs to the patient named or identified in the test requisition form.
2. All tests are performed and reported as per the turnaround time stated in the AGILUS Directory of Services.
3. Result delays could occur due to unforeseen circumstances such as non-availability of kits / equipment breakdown / natural calamities / technical downtime or any other unforeseen event.
4. A requested test might not be performed if:
 - i. Specimen received is insufficient or inappropriate
 - ii. Specimen quality is unsatisfactory
 - iii. Incorrect specimen type
 - iv. Discrepancy between identification on specimen container label and test requisition form
5. AGILUS Diagnostics confirms that all tests have been performed or assayed with highest quality standards, clinical safety & technical integrity.
6. Laboratory results should not be interpreted in isolation; it must be correlated with clinical information and be interpreted by registered medical practitioners only to determine final diagnosis.
7. Test results may vary based on time of collection, physiological condition of the patient, current medication or nutritional and dietary changes. Please consult your doctor or call us for any clarification.
8. Test results cannot be used for Medico legal purposes.
9. In case of queries please call customer care (91115 91115) within 48 hours of the report.

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